



Veterans Administration

VHA FAX TRANSMITTAL

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**Audie L. Murphy VA Hospital,
STVHCS**

7400 Merton Minter
San Antonio, TX 78229
210-617-5300

DATE:	Dec. 29, 2023
ATTN:	
FROM:	VA Office of Community Care
SUBJECT:	auth

Note:

GREEN



U.S. Department
of Veterans Affairs

**VA Form 10-7080 - Approved Referral For
Medical Care**

Veteran Name: Green, Christopher D

Veteran ICN: 1015222512V874261

Veteran EDIPI: 1225018266

Veteran Date of Birth: 1957-11-22

Gender Identity: Male

Veteran Preferred Name:

Pronoun:

Pronoun Description:

Veteran Address: 1803 E ANAQUA AVE
VICTORIA, TX 77901

Veteran Phone Number: (360)903-6290

Veteran Mobile Phone Number (if Known): (360)903-6290

Veteran Business Phone Number (If Known): (360)903-6290

Veteran Email Address (If Known): CHRISG688@COMCAST.NET

Referral Number: VA0033321803

Priority: Routine

Referral Issue Date: 2023-11-14

Expiration Date: PRELIMINARY 2024-05-12 (SEE BELOW)*

First Appointment Date: SUPPLY TO VA ASAP

Referring VA Facility: Audie L. Murphy Memorial Veterans' Hospital

VA Telephone Number: 210-949-3850

VA Fax Number:

Initial Community Care Provider/Facility: VICTORIA DERMATOLOGY PA

Initial Provider Location: VICTORIA DERMATOLOGY PA-115 Medical Dr, Victoria, TX, 77904-193200000X

Provider Name (if known): VICTORIA DERMATOLOGY PA

Community Provider NPI: 1316654221

Caregiver Type: Veteran does not rely on a caregiver

Caregiver Details:

***IN ORDER TO PROVIDE THE FULL DURATION OF THE STANDARDIZED EPISODE OF CARE (SEOC)
PLEASE CONTACT THE REFERRING VA FACILITY WITH A FIRST APPOINTMENT TO CALCULATE
A FINAL EXPIRATION DATE. CLAIM PAYMENTS ARE DEPENDENT UPON THESE DATES.**

Any claim related to this episode of care **MUST INCLUDE THE APPROVED REFERRAL NUMBER** as the
Referral Number or Prior Authorization number.

**Please see below for Additional VA Referring Facility Information, Billing Information
and Appointment/Provider Information.**

Pertinent Clinical Information

**Please view the Clinical Information in the VA Order section for more information related to the Original VA Order
Reason for Request.**

Chief Complaint: H/O MALIGNANT MELANOMA/BACK

Patient History / Clinical Findings / Diagnosis (Co-Morbidities): F/U WITH DERM REQUIRED Is this referral
for Service Connected condition? No Per VA PBM Formulary Process Management Policy from VHA Handbook
1108.08, the use of drugs solely to improve normal physiologic function or to enhance body appearance is generally
not considered medically necessary and therefore are not to be prescribed. Medications used for this purpose are

considered cosmetic. The use of medications for hair re-growth (e.g., minoxidil, finasteride, or pimecrolimus) are considered cosmetic in nature and should not be prescribed unless care is being provided in connection with the treatment of a service-connected injury. Allergies and Medications (Optional): Allergies: Patient has answered NKA Medications: Active Outpatient Medications (including Supplies): Active Outpatient Medications Status

===== 1) DICLOFENAC NA 1% TOP GEL APPLY 2 GRAMS PER DOSING ACTIVE CARD TO AFFECTED AREA FOUR TIMES A DAY AS NEEDED FOR PAIN 2) FEXOFENADINE HCL 180MG TAB TAKE ONE TABLET BY MOUTH ACTIVE EVERY MORNING (REPLACES LORATADINE) 3) FLUTICASON NASAL SPRAY 50MCG INSTILL 2 SPRAYS IN ACTIVE EACH NOSTRIL EVERY DAY FOR NASAL ALLERGIES 4) IPRAOTROPIUM BR NAS SPRAY 0.06%, 15ML USE 2 SPRAYS IN ACTIVE EACH NOSTRIL THREE TIMES A DAY AS NEEDED FOR RUNNY NOSE 5) OMEPRAZOLE 20MG EC CAP TAKE ONE CAPSULE BY MOUTH ACTIVE TWICE A DAY 1 HOUR BEFORE MEALS FOR REFLUX **DOSE CHANGE** 6) POLYETHYLENE GLYCOL 3350 ORAL PWDR TAKE 17 GRAMS(1 ACTIVE CAPFUL) BY MOUTH EVERY DAY FOR CONSTIPATION (MIX IN 8OZ OF BEVERAGE AND DRINK.) 7) SERTRALINE HCL TAB 100 MG TAKE ONE AND ONE-HALF ACTIVE TABLETS BY MOUTH EVERY DAY FOR MOOD INCREASED DOSE 8) TADALAFIL 20MG TAB TAKE 1/2 TABLET (10 MG) BY MOUTH ACTIVE AS NEEDED FOR ERECTILE DYSFUNCTION 9) TAMSULOSIN CAP 0.4MG TAKE ONE CAPSULE BY MOUTH EVERY ACTIVE DAY FOR PROSTATE

Provisional Diagnosis: Z85820 Personal history of malignant melanoma of skin

Services Authorized

The VA Order Reason for Request is the official clinical order. This scope of services associated with the medical care for this authorization is found below. Necessary services that are not included must be requested using the Request for Services procedures. Please visit the VHA Storefront www.va.gov/COMMUNITYCARE/providers/index.asp for additional resources and requirements.

Service Requested: Dermatology Comprehensive_PRCT SEOC 1.0.11

Category of Care: DERMATOLOGY

Procedural Overview - Standardized Episode of Care (SEOC)

Dermatology Comprehensive_PRCT SEOC 1.0.11 Duration: 180 Days

Description:

This authorization covers services associated with the specialty(s) identified for this episode of care, including all medical care listed below relevant to the referred care specified on the consult/referral order.

No.	Service/Procedure	Number Of Visits Authorized
1	Initial outpatient evaluation (including full body exam), treatment and follow-up visits for the referred condition on the consult/referral order	999
2	Diagnostic imaging relevant to the referred condition on the consult/referral order	999
3	Labs and pathology relevant to the referred condition on the consult/referral order	999
4	Procedures relevant to the referred condition on the consult/referral order including but not limited to: cryotherapy, biopsy (including excisional, shave, punch biopsies, etc.), excision,	999

repair, excimer laser, photodynamic therapy, UVB therapy, debridement, medically necessary hair removal

Additional Information:

- * Please visit the VHA Storefront www.va.gov/COMMUNITYCARE/providers/index.asp for additional resources and requirements pertaining to the following:
- * Pharmacy prescribing requirements
- * Durable Medical Equipment (DME), Prosthetics, and Orthotics prescribing requirements
- * Precertification (PRCT) process requirements
- * Request for Services (RFS) requirements

REFER ALL QUESTIONS RELATED TO THIS APPROVAL TO THE ISSUING VA OFFICE

Referring VA Facility: Audie L. Murphy Memorial Veterans' Hospital

Station Number: 671

Telephone Number: 210-949-3850

Address: 7400 Merton Minter Boulevard SAN ANTONIO TX 78229

Referring Provider: LISA MORENO

Referring Provider NPI: 1275625295

Unique Consult No: 671_8276596

Program Authority: Authorized/Pre-authorized VA Referral (not otherwise specified) - 1703

Affiliation: Triwest

Network: CC Network 4

Appointments/Providers Assigned to the Referral

Provider/Facility Name	Provider/Facility Location	Appt Date	Appt Time	Telephone #
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Additional Service Information

Request for Services (RFS)

A Request for Services (RFS) is a provider-generated request for new or additional care outside the scope of the current approved referral/authorization. Provider should always submit the RFS directly to the authorizing VAMC, preferably via the HSRM portal. Providers should always submit an RFS on the same day it is determined it's needed and before delivering care, unless it is emergent care. In that case, the RFS can be submitted simultaneously.

How to Submit an RFS

VA prefers providers submit an RFS via the HSRM portal, available on the VA's website

- Go to the VA Storefront at https://www.va.gov/COMMUNITYCARE/providers/Care_Coordination.asp#RFS
- Navigate to the link to the RFS form at the bottom of the section

Medical Records and Documents Requirements

Medical Records and documentation are required for all provider services. Providers are required to submit Medical documentation directly to the authorizing VAMC, preferably via upload to HSRM.

Billing and Other Referral Information

Submitting Claims

ANY CLAIMS RELATED TO THIS EPISODE OF CARE MUST BE SUBMITTED TO TRIWEST OR DELTA DENTAL AS APPROPRIATE AND INCLUDE THE APPROVED REFERRAL NUMBER.

Methods to submit claims:**Electronic Data Interchange (EDI):**

Payer ID for Medical – TWVACCN

Payer ID for Dental – CDCA1

More information on how to submit claims can be found by visiting
https://www.va.gov/COMMUNITYCARE/revenue_ops/Veteran_Care_Claims.asp

Precertification

The Standardized Episode of Care (SEOC) referral you have accepted includes certain services that require third-party payer (TPP) precertification. It is imperative that you notify VA if you have scheduled any of these specific services for a Veteran that has other health insurance (OHI); so that VA can notify the TPP. VHA is required by law to bill the TPP for care that does not have a service connection or special authority eligibility.

Notification details and specific care requiring TPP precertification for this SEOC can be found at:
https://www.va.gov/COMMUNITYCARE/providers/PRCT_requirements.asp

Community Care Medical Policies

VA Community Care Medical Policies describe standard VA health care benefits for services and procedures that community providers may recommend as necessary for a Veteran. Prior to providing care, providers should use the Community Care Medical Policies as a reference when determining if a Veteran meets VA clinical criteria. When additional services are requested, Community Care Medical Policies will be used to determine approval by a clinical reviewer. Community Care Medical Policies and supporting information can be found at: <https://www.va.gov/COMMUNITYCARE/providers/Medical-Policy.asp>

Pharmacy**Drug Safety and Administration Requirements**

- Must follow the VA National Protocol and clinical guidance for Esketamine or Ketamine administration. Ketamine treatments for mental health or for pain are not approved under a CCN referral. Prior to administration it is required the ordering and administering providers review the VA Protocol and Clinical Guidance found through the VA Formulary Search Tool available here:

- <https://www.pbm.va.gov/apps/VANationalFormulary/>

Must follow the VA Opioid Safety Guidelines and complete the required Opioid Safety Training found here:

- <https://www.pbm.va.gov/apps/VANationalFormulary/>

- https://www.va.gov/COMMUNITYCARE/providers/EDU_Training.asp

CVS Caremark is the retail pharmacy network for Veterans' immediately needed or Urgent/Emergent prescriptions.

Immediate need prescriptions:

- Must follow the VA Urgent/Emergent Formulary which can be found at <http://www.pbm.va.gov/PBM/nationalformulary.asp>
- Prescription can only go up to a 14-day supply. No refills of the immediate need medication may be authorized.
- Only a seven-day supply for opioids, or up to the opioid prescribing limit allowed by State—whichever is less—may be authorized.

Immediate need prescription extending past 14 days:

- The provider will need to send second prescription (beyond 14 days) to the referring VA medical facility's pharmacy for prescription fulfillment services.

Routine/maintenance prescriptions:

- Must be sent to the referring VA medical facility's pharmacy

If you do not have the ability to electronically submit prescriptions to pharmacies, please contact the Community Care representative at the referring VA medical facility for their pharmacy fax number. Please refer to:

https://www.va.gov/COMMUNITYCARE/providers/Service_Requirements.asp for additional instructions related to prescriptions.

Clinical Information on the VA Order

Reason for Request:

Provider to receive results: L MORENO PA-C

Type of Service: Evaluation and Treatment

Chief Complaint: H/O MALIGNANT MELANOMA/BACK

Patient History / Clinical Findings / Diagnosis (Co-Morbidities):

F/U WITH DERM REQUIRED

Is this referral for Service Connected condition? No

Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08, the

use of drugs solely to improve normal physiologic function or to enhance body

appearance is generally not considered medically necessary and therefore are not

to be prescribed. Medications used for this purpose are considered cosmetic.

The use of medications for hair re-growth (e.g., minoxidil, finasteride, or

pimecrolimus) are considered cosmetic in nature and should not be prescribed

unless care is being provided in connection with the treatment of a service-

connected injury.

Allergies and Medications (Optional):

Allergies:

Patient has answered NKA

Medications:

Active Outpatient Medications (including Supplies):

Active Outpatient Medications Status

- =====
- 1) DICLOFENAC NA 1% TOP GEL APPLY 2 GRAMS PER DOSING ACTIVE CARD TO AFFECTED AREA FOUR TIMES A DAY AS NEEDED FOR PAIN
 - 2) FEXOFENADINE HCL 180MG TAB TAKE ONE TABLET BY MOUTH ACTIVE EVERY MORNING (REPLACES LORATADINE)
 - 3) FLUTICASONE NASAL SPRAY 50MCG INSTILL 2 SPRAYS IN ACTIVE EACH NOSTRIL EVERY DAY FOR NASAL ALLERGIES
 - 4) IPRATROPIUM BR NAS SPRAY 0.06%, 15ML USE 2 SPRAYS IN ACTIVE EACH NOSTRIL THREE TIMES A DAY AS NEEDED FOR RUNNY NOSE
 - 5) OMEPRAZOLE 20MG EC CAP TAKE ONE CAPSULE BY MOUTH ACTIVE TWICE A DAY 1 HOUR BEFORE MEALS FOR REFLUX **DOSE CHANGE**
 - 6) POLYETHYLENE GLYCOL 3350 ORAL PWDR TAKE 17 GRAMS(1 ACTIVE CAPFUL) BY MOUTH EVERY DAY FOR CONSTIPATION (MIX IN

8OZ OF BEVERAGE AND DRINK.)

7) SERTRALINE HCL TAB 100 MG TAKE ONE AND ONE-HALF ACTIVE
TABLETS BY MOUTH EVERY DAY FOR MOOD INCREASED DOSE

8) TADALAFIL 20MG TAB TAKE 1/2 TABLET (10 MG) BY MOUTH ACTIVE
AS NEEDED FOR ERECTILE DYSFUNCTION

9) TAMSULOSIN CAP 0.4MG TAKE ONE CAPSULE BY MOUTH EVERY ACTIVE
DAY FOR PROSTATE

**This 10-7080 - Approved Referral For Medical Care was generated on 12/29/2023 changes made to the referral after
this date are not reflected on the form.



Veterans Health Administration

VHA Medical Documentation

Document Created: 12/29/2023 12:42

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AUDIE L MURPHY MEMORIAL VAMC 7400 Merton Minter Blvd, San Antonio, Texas 78229-4404

VA Referrals phone: 210-949-3850 Fax:

Point of Contact (POC): Quintanilla, Carlos

POC Phone: 2109493850 Fax:

Patient Name: GREEN,CHRISTOPHER D DOB: 11/22/1957

1803 E ANAQUA AVE
VICTORIA, TEXAS 77901

Phone (residential): 360 903 6290
Phone (mobile): 360 903 6290

Referral Type: Community Care
Network

Next of Kin contact information

MATTHEWS,CONNIE

Address (Next Of Kin)
PO BOX 5616
CHARLESTON, OREGON 97420

Phone: (360)200-2890

CONSULTS

Current PC Provider: MORENO,LISA ANN
Current PC Team: VICTORIA GREEN TEAM *WH* *CRH*
Current Pat. Status: Outpatient
Primary Eligibility: SC LESS THAN 50%
UCID: 671_8276596
Patient Type:
OEF/OIF: NO

Order Information

To Service: COMMUNITY CARE-DERMATOLOGY
Attention:
From Service: VOP PHONE PACT NURSE GREEN
Requesting Provider: MORENO,LISA ANN
Service is to be rendered on an OUTPATIENT basis
Place: Consultant's choice
Urgency: Routine
Clinically Ind. Date: 2023-11-13 00:00:00

DST ID:

Orderable Item:

Consult: Consult

Provisional Diagnosis: Personal History Of Malignant Melanoma Of Skin (ICD-10-CM Z85.820)

Reason For Request:

Provider to receive results: L MORENO PA-C

Type of Service: Evaluation and Treatment

Chief Complaint: H/O MALIGNANT MELANOMA/BACK

Patient History / Clinical Findings / Diagnosis (Co-Morbidities):

F/U WITH DERM REQUIRED

Is this referral for Service Connected condition? No

Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08, the use of drugs solely to improve normal physiologic function or to enhance body appearance is generally not considered medically necessary and therefore are not to be prescribed. Medications used for this purpose are considered cosmetic. The use of medications for hair re-growth (e.g., minoxidil, finasteride, or pimecrolimus) are considered cosmetic in nature and should not be prescribed unless care is being provided in connection with the treatment of a service-connected injury. Allergies and Medications (Optional): Allergies: Patient has answered NKA Medications:

Active Outpatient Medications (including Supplies):

Active Outpatient Medications	Status
===== 1) DICLOFENAC NA	
1% TOP GEL APPLY 2 GRAMS PER DOSING	ACTIVE
CARD TO AFFECTED AREA FOUR TIMES A DAY AS NEEDED	
FOR PAIN 2) FEXOFENADINE HCL 180MG TAB TAKE ONE TABLET BY MOUTH	ACTIVE
EVERY MORNING (REPLACES LORATADINE) 3) FLUTICASONE NASAL SPRAY 50MCG INSTILL 2 SPRAYS IN	
ACTIVE	
EACH NOSTRIL EVERY DAY FOR NASAL ALLERGIES 4) IPRATROPIUM BR NAS SPRAY 0.06%, 15ML USE 2	
SPRAYS IN	ACTIVE
EACH NOSTRIL THREE TIMES A DAY AS NEEDED FOR RUNNY	
NOSE 5) OMEPRAZOLE 20MG EC CAP TAKE ONE CAPSULE BY MOUTH	ACTIVE
TWICE A DAY 1 HOUR BEFORE MEALS FOR REFLUX **DOSE	
CHANGE** 6) POLYETHYLENE GLYCOL 3350 ORAL PWDR TAKE 17 GRAMS(1	ACTIVE
CAPFUL) BY MOUTH EVERY DAY FOR CONSTIPATION (MIX IN	
8OZ OF BEVERAGE AND DRINK.) 7) SERTRALINE HCL TAB 100 MG TAKE ONE AND ONE-HALF	ACTIVE
TABLETS BY MOUTH EVERY DAY FOR MOOD INCREASED DOSE 8) TADALAFIL 20MG TAB TAKE 1/2	
TABLET (10 MG) BY MOUTH	ACTIVE
AS NEEDED FOR ERECTILE DYSFUNCTION 9) TAMSULOSIN CAP 0.4MG TAKE ONE CAPSULE BY MOUTH	
EVERY	ACTIVE
DAY FOR PROSTATE	

Inter-facility Information

AUDIE L. MURPHY MEMORIAL HOSP

Status: ACTIVE

Last Action: RECEIVED

Facility

Activity Date/Time/Zone Responsible Person Entered By

CPRS RELEASED ORDER	11/13/23 22:01	MORENO,LISA ANN	MORENO,LISA ANN
ADDED COMMENT	11/13/23 00:00	MORENO,LISA ANN	MORENO,LISA ANN
RECEIVED	11/14/23 15:44	DORSEY,MERTHA V	DORSEY,MERTHA V

Note: TIME ZONE is local if not indicated

No local TIU results or Medicine results available for this consult

=====

===== END =====

PROBLEM LIST

Sensitive Diagnoses

No sensitive diagnoses were provided.

Other Diagnoses

Problem	Code
Allergic rhinitis, unspecified	J30.9
Anxiety disorder, unspecified	F41.9
Benign prostatic hyperplasia with lower urinary tr	N40.1
Chronic rhinitis	J31.0
Contact with and exposure to other hazardous subst	Z77.29
Diarrhea, unspecified	R19.7
Elevated blood-pressure reading, w/o diagnosis of	R03.0
Gastro-esophageal reflux disease without esophagit	K21.9
Male erectile dysfunction, unspecified	N52.9
Obstructive sleep apnea (adult) (pediatric)	G47.33
Personal history of malignant melanoma of skin	Z85.820
Tobacco use	Z72.0

MEDICATIONS

100 most recent outpatient medications released by VA to Veteran in the last 6 months

Medication Name and Dose	Quantity	Refill Number	Issue and Fill Date	Status
DICLOFENAC NA 1% TOP GEL APPLY 2 GRAMS PER DOSING CARD TO AFFECTED AREA FOUR TIMES A DAY AS NEEDED FOR PAIN	Qty:100	Fill: 1 of 4	Orig: 2023-08-21 Last: 2023-08-21	ACTIVE
FERROUS GLUCONATE 324-325MG TAB TAKE ONE TABLET BY MOUTH EVERY DAY	Qty:100	Fill: 1 of 0	Orig: 2023-12-08 Last: 2023-12-08	ACTIVE
FLUTICASONE NASAL SPRAY 50MCG INSTILL 2 SPRAYS IN EACH NOSTRIL EVERY DAY FOR NASAL ALLERGIES	Qty:1	Fill: 2 of 5	Orig: 2023-08-25 Last: 2023-11-29	ACTIVE
TAMSULOSIN HCL 0.4MG CAP TAKE ONE CAPSULE BY MOUTH EVERY DAY FOR PROSTATE	Qty:90	Fill: 1 of 3	Orig: 2023-12-08 Last: 2023-12-11	ACTIVE
LORATADINE TAB 10MG TAKE ONE TABLET BY MOUTH EVERY DAY FOR ALLERGIES.	Qty:90	Fill: 1 of 3	Orig: 2023-06-08 Last: 2023-09-28	DISCONTINUED
TADALAFIL 10MG TAB TAKE ONE TABLET BY MOUTH EVERY DAY AS NEEDED FOR ERECTILE DYSFUNCTION	Qty:6	Fill: 1 of 11	Orig: 2023-06-08 Last: 2023-07-21	DISCONTINUED
FEXOFENADINE HCL 180MG TAB TAKE ONE TABLET BY MOUTH EVERY MORNING (REPLACES LORATADINE)	Qty:90	Fill: 1 of 0	Orig: 2023-12-08 Last: 2024-01-25	SUSPENDED

OMEPRAZOLE 20MG EC CAP	Qty:180	Fill: 1 of 3	Orig: 2023-12-08	SUSPENDED
TAKE ONE CAPSULE BY MOUTH TWICE A DAY 1 HOUR BEFORE MEALS FOR REFLUX **DOSE CHANGE**			Last: 2024-01-27	
SERTRALINE HCL TAB 100 MG	Qty:135	Fill: 1 of 3	Orig: 2023-12-08	SUSPENDED
TAKE ONE AND ONE-HALF TABLETS BY MOUTH EVERY DAY FOR MOOD INCREASED DOSE			Last: 2024-02-29	
TADALAFIL 20MG TAB	Qty:3	Fill: 6 of 10	Orig: 2023-07-28	SUSPENDED
TAKE 1/2 TABLET (10 MG) BY MOUTH AS NEEDED FOR ERECTILE DYSFUNCTION			Last: 2024-01-14	

ALLERGIES

No documented allergies

PROGRESS NOTES

LOCAL TITLE: PRIMARY CARE NURSE TELEPHONE ENCOUNTER NOTE
 STANDARD TITLE: PRIMARY CARE TELEPHONE ENCOUNTER NOTE
 DATE OF NOTE: NOV 13, 2023@08:50 ENTRY DATE: NOV 13, 2023@08:57:57
 AUTHOR: JENNINGS,KAREN J EXP COSIGNER:
 URGENCY: STATUS: COMPLETED

*** PRIMARY CARE NURSE TELEPHONE ENCOUNTER NOTE Has ADDENDA ***

Primary Care Nurse Message Manager Return Telephone Call
 GREEN,CHRISTOPHER D, 554-98-4968 Age: 65 Sex: MALE Race: DECLINED TO ANSWER

Date and Time of call: Nov 13,2023@08:50

Provider assigned to patient Moreno PA

Diagnosis: Vaccines and CC Derm

Callers Telephone Number PATIENT PHONE - 360 903 6290

Spoke with Patient

Reason for call: "Veteran is due for annual vaccines and a dermatology appointment. Please call.

Phone: 360-903-6290."

Call to patient who is requesting his annual flu and whatever other vaccines he is due for. RN offered the day of his lab appointment, the day of his upcoming provider appointment, or sooner. He would like to come 11/20 for flu and P20 vaccines. RN placed RTC and asked Team MSA via MM to call and get the time confirmed.

As for the CC Derm consult, RN reminded him it was discontinued last December. He shares he only needs to go annually thus he didn't feel he needed at that time. He is now requesting.

*PA Moreno, patient asking you re-submit the CC Derm consult.

/es/ Karen Jennings, RN, BSN

Staff Nurse

Signed: 11/13/2023 09:02

Receipt Acknowledged By:

11/13/2023 16:35 /es/ LISA ANN MORENO
PHYSICIAN ASSISTANT

11/13/2023 ADDENDUM STATUS: COMPLETED

last pcg note says veteran has h/o melanoma on back.

Plan: re-submit CC derm consult that veteran declined

/es/ LISA ANN MORENO
PHYSICIAN ASSISTANT
Signed: 11/13/2023 22:01

Receipt Acknowledged By:

* AWAITING SIGNATURE * JENNINGS,KAREN J

Copy/ Paste Legal Disclaimer: This section of the REFDOC package has been copy/pasted from the patient's electronic health record and may not represent the entire contents of the record.

LOCAL TITLE: VOPC TELEHEALTH ESTABLISHED

STANDARD TITLE: TELEHEALTH NOTE

DATE OF NOTE: JUN 08, 2023@09:42 ENTRY DATE: JUN 08, 2023@06:27:13

AUTHOR: MORENO,LISA ANN EXP COSIGNER:

URGENCY: STATUS: COMPLETED

*** VOPC TELEHEALTH ESTABLISHED Has ADDENDA ***

*** PATIENT CONSENTED TO CLINIC VISIT VIA TELEHEALTH ***

Telehealth Disclosure: Visit conducted by clinical video telehealth.

Patient verbal consent obtained.

Veteran was identified using full name, SSN, and date of birth.

CHIEF COMPLAINT: Evaluation of chronic medical problems.

HISTORY OF PRESENT ILLNESS:

- Vet c/o abd bloating, nausea, and frequent BM x 7mo, feels like incomplete emptying because every time he eats/drinks has urge to have BM. Nausea is resolved with sneezing. States this was discussed with CC ENT who could not explain this. No vomiting. Denies black or bloody stools. Describes loose BMs that are frequent, only occas. formed stools. Does not avoid dairy, never tried dairy free diet. Takes PPI for GERD. Denies dysphagia.
- Vet states he is scheduled for a sinus laser procedure/endoscopy w/ENT doctor. Was told he had deviated septum. He is using fluticasone nasal spray, states azelastine dries out his nose too much. Sleep clinic referred to ENT. Vet states he got new CPAP machine but did not get teaching as they wanted him to drive to ALM.
- would like to increase sertraline dose, desires better motivation, less anhedonia.
- ED- Viagra caused nasal stuffiness, would like to try alternative. ? SSRI related. Reports good libido.
- last Colonoscopy 5 years ago, out of state. No prior EGD.
- reports had epidose with LUQ abdom "bulging" and pain and LUQ still "sore".

ALLERGIES:

Patient has answered NKA

MEDICATIONS:

Active Outpatient Medications (including Supplies):

FLUTICASONE NASAL SPRAY 50MCG INSTILL 2 SPRAYS IN EACH NOSTRIL EVERY DAY FOR NASAL ALLERGIES ACTIVE

IPRATROPIUM BR NAS SPRAY 0.06%, 15ML USE 2 SPRAYS IN EACH NOSTRIL THREE TIMES A DAY AS NEEDED FOR RUNNY NOSE ACTIVE

LORATADINE TAB 10MG TAKE ONE TABLET BY MOUTH EVERY DAY FOR ALLERGIES. ACTIVE

OMEPRAZOLE 20MG EC CAP TAKE ONE CAPSULE BY MOUTH EVERY MORNING BEFORE MEAL FOR REFLUX. TAKE 30 TO 60 MINUTES BEFORE MEAL. ACTIVE

POLYETHYLENE GLYCOL 3350 ORAL PWDR TAKE 17 GRAMS(1 CAPFUL) BY MOUTH EVERY DAY FOR CONSTIPATION (MIX IN 8OZ OF BEVERAGE AND DRINK.) ACTIVE

*** not taking

SERTRALINE HCL TAB 100 MG TAKE ONE TABLET BY MOUTH EVERY DAY FOR MOOD ACTIVE

TAMSULOSIN CAP 0.4MG TAKE ONE CAPSULE BY MOUTH EVERY DAY FOR PROSTATE ACTIVE

OTC/HERBAL MEDICATIONS:

NON-VA PRESCRIPTION MEDICATIONS:

Medication Reconciliation:

Medication Reconciliation includes comparing the VA list of all medications to the patient's/caregiver's medication information:

I have reviewed this list of medications with the Patient/caregiver and medication reconciliation was performed. Discrepancies were noted with ***** above.

PERTINENT REVIEW OF SYSTEMS: wgt up 25lbs since 12/2022, states usual efforts at gym and healthy eating not helping for wgt loss efforts.

PHYSICAL EXAM: (portions of exam completed by trained CVT Telepresenter under provider guidance/direction)

Vitals: SEE COVER SHEET

GENERAL: No acute distress, well developed, obese, ambulatory

HEENT: Normocephalic, atraumatic.

CHEST/LUNGS: Clear to auscultation bilaterally

HEART/VASCULAR: Regular rate, regular rhythm, no rub, no gallop, no murmur

NEUROLOGIC: Mental status: alert and oriented. No significant abnormality of gait appreciated.

LABS REVIEWED: 6/1/23 labs

ASSESSMENT/PLAN:

1. bloating/nausea/diarrhea/LUQ abdom pain- CC Abdom US, CC GI consult. Trial of dicyclomine 10mg qid prior to meals. Trial dairy-free diet x 2 wks.

- no improvement with Miralax

- on PPI for GERD

2. GERD without dysphagia- cont. omeprazole 20mg, reviewed lifestyle modifications. Vet reports better sx control on PPI than on H2RA. See #1.

- No h/o GI bleed, PUD, esoph stricture. No prior EGD.

3. sinus/allergic rhinitis- f/u CC ENT planned for endoscopic eval and possible laser procedure. cont. loratadine + fluticasone.

4. BPH/LUTS s/p Rezum steam therapy 06/2020- cont. tamsulosin

5. anxiety/cannabis dependence- INCREASE sertraline from 100mg to 150mg daily. Goal for improved motivation, reduced anhedonia. f/u phone appt 1 mo. no safety concerns.

6. obstructive sleep apnea- f/u VA sleep clinic for teaching on new CPAP. Working on nasal congestion/sinus with ENT. PCP recommended 5% wgt loss, avoid sleeping supine, avoid ETOH and other sedative prior to bed.
- significant wgt gain 25lbs contributes

7. h/o melanoma/back (> 10 yrs ago)- f/u CC derm

8. ED- trial of tadalafil

- could not tolerate increasing sildenafil-> nasal congestion

9. unexplained wgt gain/borderline TSH- f/u TSH 1 mo.

HCM: colonoscopy (Washington VA?) at age 60 yrs per veteran, no polyps per veteran. No Fmhx of colon CA. No record in JLV or VISTA IMAGING.

IMAGING: CC ABdom US

CONSULTS: CC GI,

Return to Clinic: 1 mo lab/ 1mo phone PCP, 6mo CVT w nonfasting lab

Time spent on this face to face encounter: 30 minutes with at least 50% of that time spent on counseling, teaching, and collaboration of care.

CLINICAL REMINDER ACTIVITY

Follow-up Pos Alcohol :

Patient's AUDIT-C score was greater than or equal to 5; brief alcohol intervention is indicated.

Shared concern that the patient may be drinking at unhealthy levels known to increase his/her risk of alcohol related health problems.

Specifically the following were reviewed:

High blood pressure, heart disease, liver disease, seizures, medication interactions, depression, anxiety, bleeding from the stomach

The patient was advised/informed to drink within safe limits, which are no more than 2 drinks per day on average and no more than 4 drinks on any one day AND no more than 14 drinks per week. Will discuss again at next visit.

Avg Risk Colorectal Cancer Screen:

AVERAGE RISK colorectal cancer screening is due based on information available to this clinical reminder

FOBT/FIT (Fecal Immunochemical Testing) has been ordered. See order tab for details.

/es/ LISA ANN MORENO
PHYSICIAN ASSISTANT
Signed: 06/08/2023 11:26

11/14/2023 ADDENDUM STATUS: COMPLETED
Team had requested records from DeTar of a colonoscopy but that facility faxed back stating they had no record of having ever seen this patient.

/es/ Karen Jennings, RN, BSN
Staff Nurse
Signed: 11/14/2023 13:44

LOCAL TITLE: PRIMARY CARE NURSE NEW PATIENT/ANNUAL NOTE
STANDARD TITLE: PRIMARY CARE NURSING NOTE
DATE OF NOTE: JUN 07, 2023@11:41 ENTRY DATE: JUN 07, 2023@11:41:45
AUTHOR: GAMEZ,CANDICE M EXP COSIGNER:
URGENCY: STATUS: COMPLETED

*** PRIMARY CARE NURSE NEW PATIENT/ANNUAL NOTE Has ADDENDA ***

Annual Note
GREEN,CHRISTOPHER D, 554-98-4968 Age:65 Sex:MALE
Race: DECLINED TO ANSWER

Problem/Chief Complaint: Prescreen nursing note and clinical reminders completed. Advised vet to arrive 30 min prior to appt time. PVU.

-F/U labs

-Vet will bring COVID card to update vaccines. Shingles vaccine x 2 doses already taken and refused PCV20 vaccine.

-Vet c/o abd bloating, nausea, and diarrhea x 1 year.

-Vet states he is scheduled for a sinuplasty w/ENT doctor and he would like 2nd opinion from PCP.

-Colonoscopy 5 years ago

Has the patient Traveled Outside the United States in the last 21 days?
No

Coronavirus Disease 2019 (COVID-19) Screen
The patient was asked if in the last 14 days they have had new onset of any COVID-19 symptoms. They report the following:

No symptoms

Within the past 14 days, the patient reports no exposure to someone with a febrile/respiratory illness or someone with a known or suspected case of COVID-19 (within 6 feet for > 15 minutes).

Result:

Screen is negative.

Patient reports prior COVID-19 Diagnosis.
Comment: 2021

Current Vital Signs: Ht: 71 in [180.3 cm] (06/27/2022 15:00)

Wt: 205 lb [92.99 kg] (12/07/2022 14:08)

Temp: 97.9 F [36.6 C] (12/07/2022 14:08)

Puls: 78 (12/07/2022 14:08)

Resp: 18 (12/07/2022 14:08)

BP: 146/75 (12/07/2022 14:12)

BMI: 28.7

Pain Scale: 0 (12/07/2022 14:08)

Pain Assessment:

New Pain or Unacceptable Pain

Patient denies pain, Pain equals 0

0-10 NRS reviewed. Patient communicates understanding.

Patient Education Assessment:

Have your education needs changed in the last 12 months? No

Screening for suspected abuse and neglect.

Not suspected

Screen Results

Abuse and Neglect screen negative.

Nutrition:

No problem

Skin Integrity Screen

Is patient wheelchair bound, needs assistance with transfers, or has existing pressure ulcers?

No

Functional Assessment:

Is Veteran 74 or greater?

No

Is patient independent with ADLs: bathing, dressing, grooming, toileting, feeding or ambulation?

Yes

Have you had a fall in the past 12 months?

No

The patient/caregiver was provided the "Patient Information Handbook" and information on Methicillin-resistant Staphylococcus Aureus (MRSA) and Multi-drug Resistant Organisms (MDROs); hand hygiene; respiratory etiquette; exercise and nutrition; cancer screening; pain management; tobacco cessation; alcohol use; speak up information; and advance directives.

Since your last visit have there been any life or family issues that cause you to be worried or stressed that you feel you would like to speak with a mental health counselor about?

No

CLINICAL REMINDER ACTIVITY

Advance Directive Screening:

Veteran has been educated about advance directives and will contact the social worker when ready to complete a Advance Directive.

Alcohol Use Screen (AUDIT-C):

Alcohol Screen:

SCREEN FOR ALCOHOL (AUDIT-C)

An alcohol screening test (AUDIT-C) was positive (score=5).

1. How often did you have a drink containing alcohol in the past year?

Four or more times a week

2. How many drinks containing alcohol did you have on a typical day when you were drinking in the past year?

Three or four drinks

3. How often did you have six or more drinks on one occasion in the past year?

Never

Depression Screening:

Perform PHQ-2

A PHQ-2 screen was performed. The score was 0 which is a negative screen for depression.

Over the past two weeks, how often have you been bothered by the following problems?

1. Little interest or pleasure in doing things

Not at all

2. Feeling down, depressed, or hopeless

Not at all

STX ORGAN DONOR:

ORGAN DONOR STATUS

Patient is already an organ donor or wants to be an organ donor.

Suicide Screen:

C-SSRS Screening

Columbia Suicide Severity Rating Scale (C-SSRS) screener

1. Over the past month, have you wished you were dead or wished you could go to sleep and not wake up?

No

2. Over the past month, have you had any actual thoughts of killing yourself?

No

3. Over the past month, have you been thinking about how you might do this?

Response not required due to responses to other questions.

4. Over the past month, have you had these thoughts and had some intention of acting on them?

Response not required due to responses to other questions.

5. Over the past month, have you started to work out or worked out the details of how to kill yourself?

Response not required due to responses to other questions.

6. If yes, at any time in the past month did you intend to carry out this plan?

Response not required due to responses to other questions.

7. In your lifetime, have you ever done anything, started to do anything, or prepared to do anything to end your life (for example, collected pills, obtained a gun, gave away valuables, went to the roof but didn't jump)?

No

8. If YES, was this within the past 3 months?

Response not required due to responses to other questions.

Tobacco Use Screening:

The patient is a former tobacco user.

The patient quit less than one year ago.

Annual Nursing Health Assessment:

Patient had annual adult health history exam at this encounter.

Comment: prescreen

Herpes Zoster (Shingles) Vaccine:

The patient declines to receive the recommended dose of zoster (shingles) vaccine.

Immunization: ZOSTER RECOMBINANT

Refusal Reason: OTHER

Patient refuses all immunization(s) in the ZOSTER group

Comment: vet had both doses of Shingrix vaccines

Date Documented: 06/07/23 11:53

Pneumococcal Conjugate Vaccine (PCV15/PCV20):

Refuses PCV vaccine

Immunization: PNEUMOCOCCAL CONJUGATE, UNSPECIFIED FORMULATION

Refusal Reason: PATIENT DECISION

Patient refuses all immunization(s) in the PneumoPCV group

Date Documented: 06/07/23 11:54

/es/ CANDICE M. GAMEZ

Signed: 06/07/2023 12:01

Receipt Acknowledged By:

06/07/2023 12:51 /es/ LISA A WARD

Assistant Nurse Manager

06/08/2023 ADDENDUM STATUS: COMPLETED

Temp: 97.2 F [36.2 C] (06/08/2023 09:29)

P: 88 (06/08/2023 09:29)

R: 18 (06/08/2023 09:29)

B/P: 134/68 (06/08/2023 09:29)

Weight: 231.5 lb [105.01 kg] (06/08/2023 09:29)

Height: 71 in [180.3 cm] (06/08/2023 09:29)

BMI:32.4

CLINICAL REMINDER ACTIVITY

COVID-19 Immunization Primary Series:

Moderna

Patient received a prior dose of the Moderna COVID-19 Vaccine.

Documented: COVID-19 (MODERNA), MRNA, LNP-S, PF, 100 MCG/0.5ML DOSE OR 50 MCG/0.25ML DOSE

Historical Date Administered: Apr 22, 2021 Series: Series 1
Outside Location: FHC
Information Source: FROM PATIENT'S WRITTEN RECORD

Patient received a prior dose of the Moderna COVID-19 Vaccine.
Documented: COVID-19 (MODERNA), MRNA, LNP-S, PF, 100 MCG/0.5ML DOSE OR 50
MCG/0.25ML DOSE

Historical Date Administered: May 20, 2021 Series: Series 2
Outside Location: fhc
Information Source: FROM PATIENT'S WRITTEN RECORD

Patient received a prior dose of the Moderna COVID-19 Vaccine.
Documented: COVID-19 (MODERNA), MRNA, LNP-S, PF, 100 MCG/0.5ML DOSE OR 50
MCG/0.25ML DOSE

Historical Date Administered: Dec 17, 2021 Series: Series 3
Outside Location: NOVANT HEALTH
Information Source: FROM PATIENT'S WRITTEN RECORD

/es/ CYNTHIA G. CAMPBELL

LVN

Signed: 06/08/2023 09:36

 Department of Veterans Affairs		REQUEST FOR SERVICES (RFS) FORM	
PREVIOUS AUTHORIZATION NUMBER:		NOTE: The Request for Services (RFS) Form 10-10172 must be submitted via an approved method (HSRM, Electronic fax, Direct Messaging, traditional fax, or mail). Completion of this form is REQUIRED and MUST BE SIGNED by the requesting provider for further care to be rendered to a Veteran patient.	
TODAY'S DATE (MM/DD/YYYY):			
SECTION I: VETERAN INFORMATION			
1. VETERAN'S LEGAL FULL NAME (First, MI, Last):		2. DOB (MM/DD/YYYY):	
3. VA FACILITY: STVHCS/Pink Team - Team Fax: (210) 443-0301		4. VA LOCATION: San Antonio, TX Ph: (210) 949-3850	
SECTION II: ORDERING PROVIDER INFORMATION			
5. REQUESTING PROVIDER'S NAME:		6. NPI #:	7. SPECIALTY:
8. OFFICE NAME & ADDRESS:			
9. SECURE EMAIL ADDRESS:			
10. PHONE NUMBER:		11. FAX NUMBER:	☐ 12. INDIAN HEALTH SERVICES (IHS) PROVIDER?
SECTION III: TYPE OF CARE REQUEST			
13. PLEASE INDICATE CLINICAL URGENCY (<i>Urgent care is only applicable for requests that require less than 3 days to process. If care is needed within 48 hours or if Veteran is at risk for Suicide/Homicide, please call the VA directly on the same day as completed RFS form submission. Do NOT mark urgent for administrative urgency:</i>) ☐ ROUTINE ☐ URGENT			
14. DIAGNOSIS (ICD-10 Code/Description):		15. DATE OF SERVICE (MM/DD/YYYY) &/OR ANTICIPATED LENGTH OF CARE:	
16. CPT/HCPCS CODE &/OR DESCRIPTION OF REQUESTED SERVICES (<i>Include units/visits, add second list page, if needed:</i>)			
17. HOW MANY VISITS HAVE OCCURRED SO FAR? (If known)		18. IS THIS A REFERRAL TO ANOTHER SPECIALTY? ☐ YES (<i>If "YES," please fill out the Servicing Provider/Specialty information below</i>) ☐ NO	
19. SERVICING PROVIDER'S NAME:		20. NPI #:	21. SPECIALTY:
22. OFFICE NAME & ADDRESS:			
23. SECURE EMAIL ADDRESS:			
24. PHONE NUMBER:		25. FAX NUMBER:	
SECTION IV: TYPE OF SERVICE REQUESTED			
26. OUTPATIENT CARE: ☐ PT ☐ OT ☐ SPEECH THERAPY FREQUENCY & DURATION:		27. SURGICAL PROCEDURE: ☐ INPATIENT ☐ OUTPATIENT FACILITY NAME:	
28. ☐ IN-OFFICE PROCEDURE		29. INPATIENT CARE: ☐ LTACH ☐ ACUTE REHAB ☐ BH	
30. ☐ ADDITIONAL OFFICE VISITS (List # needed):		31. ☐ EXTENSION OF VALIDITY DATES	
32. ☐ EMERGENCY ROOM CARE		33. ☐ LABS (<i>If done outside of office, please provide facility above</i>)	
34. ☐ RADIOLOGY/IMAGING (<i>If done outside of office, please provide facility above</i>)		35. ☐ PRE-OPS LABS ☐ CHEST XRAY ☐ EKG ☐ OTHER:	
36. JUSTIFICATION FOR REQUEST (<i>To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested.</i>)			

VETERAN'S LEGAL FULL NAME (First, MI, Last):		
SECTION V: GERIATRICS AND EXTENDED CARE SERVICES (If applicable)		
37. ☐ COMMUNITY ADULT DAY HEALTH CARE ☐ COMMUNITY NURSING HOME ☐ HOMEMAKER/HOME HEALTH AIDE ☐ HOME INFUSION ☐ HOSPICE/PALLIATIVE CARE ☐ RESPITE ☐ SKILLED HOME HEALTH CARE ☐ OTHER: _____		
FREQUENCY & DURATION: _____		
38. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).		
SECTION VI: HOME OXYGEN INFORMATION (If applicable)		
39. PAO2 AT REST:	40. O2 SAT AT REST:	41. OXYGEN FLOW RATE:
42. EXTENT OF SUPPORT (Continuous, Intermittent, Specific Activity):		
43. OXYGEN EQUIPMENT (Stationary/Portable):		
44. DELIVERY SYSTEM (Cannula, Mask, Other):		
SECTION VII: DME & PROSTHETICS INFORMATION (If applicable)		
45. HCPS FOR THE ITEM(S) BEING PRESCRIBED:		
46. BRAND, MAKE, MODEL, PART NUMBERS:		
47. MEASUREMENTS:		
48. QUANTITY:	49. ICD-10:	50. PROVISIONAL DIAGNOSIS:
51. DELIVERY/PICKUP OPTIONS:		
☐ DELIVER TO ORDERING PROVIDER'S ADDRESS ☐ VETERAN WILL PICKUP AT THE VA MEDICAL CENTER ☐ DELIVER TO COMMUNITY VENDOR FOR DELIVERY & SETUP FOR DME ☐ DELIVER TO VETERAN'S HOME		
SECTION VIII: DURABLE MEDICAL EQUIPMENT (DME) EDUCATION & TRAINING (If applicable)		
Please see DME Requirements/Pharmacy Requirements - Community Care (va.gov) for URGENT DME requests.		
NOTE: Failure to thoroughly complete the RFS for DME will result in delayed patient care & prevent the VA from DME fulfillment.		
52. BEFORE DME WILL BE ISSUED, EDUCATION, TRAINING, &/OR FITTING OF DME (as applicable for the specific DME being ordered) TO THE VETERAN MUST BE COMPLETE. PLEASE INDICATE WHETHER THE FOLLOWING HAS BEEN COMPLETED FOR THE VETERAN. NOTE: If not completed, DME will be mailed to requesting provider's address to coordinate an alternative time for proper instruction on DME use.		A. EDUCATION: ☐ YES ☐ NO B. TRAINING: ☐ YES ☐ NO ☐ N/A C. FITTING: ☐ YES ☐ NO ☐ N/A
53. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).		

VETERAN'S LEGAL FULL NAME (First, MI, Last):	
SECTION IX: THERAPEUTIC FOOTWEAR ASSESSMENT INFORMATION (If applicable)	
54. FILL OUT THE INFORMATION BELOW (If applicable): ☐ LEFT FOOT ☐ RIGHT FOOT ☐ BILATERAL ☐ PREFABRICATED THERAPEUTIC FOOTWEAR ☐ CUSTOM THERAPEUTIC FOOTWEAR	NOTE: For prescription of therapeutic footwear due to disease pathology resulting in neuropathy or peripheral artery disease.
NOTE: For prescription of therapeutic footwear for severe or gross foot deformity which cannot be accommodated with conventional footwear.	55. CHECK APPROPRIATE DIABETIC/AMPUTATION RISK SCORE: ☐ RISK SCORE 2: PATIENT DEMONSTRATED SENSORY LOSS (<i>inability to perceive the Semmes-Weinstein 5.07 monofilament</i>), DIMINISHED CIRCULATION AS EVIDENCED BY ABSENT OR WEAKLY PALPABLE PULSES, FOOT DEFORMITY, OR MINOR FOOT INFECTION, & A DIAGNOSIS OF DIABETES.
DESCRIBE FOOT DEFORMITY AND ADDITIONAL DETAILS:	☐ RISK SCORE 3: PATIENT DEMONSTRATED PERIPHERAL NEUROPATHY WITH SENSORY LOSS (<i>i.e., inability to perceive the Semmes-Weinstein 5.07 monofilament</i>), AND DIMINISHED CIRCULATION, AND FOOT DEFORMITY, OR MINOR FOOT INFECTION & A DIAGNOSIS OF DIABETES, OR ANY OF THE FOLLOWING BY ITSELF: (1) PRIOR ULCER, OSTEOMYELITIS OR HISTORY OF PRIOR AMPUTATION; (2) SEVERE PERIPHERAL VASCULAR DISEASE (PVD) (<i>intermittent claudication, dependent rubor with pallor on elevation, or critical limb ischemia manifested by rest pain, ulceration or gangrene</i>); (3) CHARCOT'S JOINT DISEASE WITH FOOT DEFORMITY; & (4) END STAGE RENAL DISEASE.
*ATTESTATION: I do hereby attest that the forgoing information is true, accurate, & complete to the best of my knowledge & I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability.	
I do hereby acknowledge that VA reserves the right to perform the requested service(s) if the following criteria are met: (1) The patient agrees to receive services from VA (2) Service(s) are available at VA facility & are able to be provided by the clinically indicated date (3) It is determined to be within the patients best interest. Upon completion of the requested service(s), VA will provide all resulting medical documentation to the ordering provider. If all criteria listed are not true & VA agrees the service(s) are clinically indicated, VA will provide a referral for services to be performed in the community.	
I do hereby attest that upon receipt of order/consult results, I will assume responsibility for reviewing said results, addressing significant findings, & providing continued care.	
56. REQUESTING PROVIDER SIGNATURE (Required):	57. TODAY'S DATE (MM/DD/YYYY):

To facilitate timely review of this request, the most recent office notes & plan of care must accompany this signed form.

For more information please visit: https://www.va.gov/COMMUNITYCARE/providers/Care_Coordination.asp.

VA Community Care Medical Policies describe standard VA health care benefit for services and procedures that community providers may recommend as necessary for a Veteran. Prior to providing care, providers should use the Community Care Medical Policies as a reference when determining if a Veteran meets VA clinical criteria. When additional services are requested, Community Care Medical Policies will be used to determine approval by a clinical reviewer. Community Care Medical Policies & supporting information can be found at: <https://www.va.gov/COMMUNITYCARE/providers/Medical-Policy.asp>